Plan: PLAN-A | Drugs: Ozempic, Trulicity | Version: 2026-01 | Effective: 2026-01-01

PLAN-A Pharmacy Policy: GLP-1 Receptor Agonists

Overview

This policy describes how PLAN-A reviews pharmacy benefit requests for glucagon-like peptide-1 (GLP-1) receptor agonist medications. Its purpose is to give members, prescribers, and reviewers a single, consistent reference for what the plan requires before these medications are approved, so that decisions are predictable and can be made promptly.

The policy covers Ozempic (semaglutide) and Trulicity (dulaglutide). Prior Authorization (PA) is required for both drugs. A request submitted without a PA on file will not be paid at the pharmacy counter, and the pharmacy will be told to contact the prescriber.

Reviews are performed by plan pharmacists and clinical reviewers who apply the criteria below to the information submitted. Reviewers may contact the prescriber for clarification, and standard requests are normally decided within several business days of receiving a complete submission. Expedited review is available when a delay could seriously affect the member's health, and the prescriber should state this clearly on the request.

Coverage Criteria

A request is considered for approval when all of the following conditions are met.

- The member is an adult with documented type 2 diabetes.

- The medication is not requested for weight loss alone. Requests made solely for weight management are not covered under this policy.

- The member is not receiving two GLP-1 agents at the same time. A request for concurrent Ozempic and Trulicity is pended until the prescriber confirms which drug will be stopped.

- The medication is prescribed by the treating clinician who is managing the member's diabetes care.

Meeting these criteria does not guarantee approval. Reviewers also confirm that the member is actively enrolled in the plan on the date of the request, that the prescription is valid, and that the information supplied is internally consistent. Requests that conflict with the clinical notes, such as a dose that does not match the documented treatment plan, may be returned to the prescriber for correction before a decision is made.

Required Documentation

Every PA request must include the items listed below. The prescriber's office should attach them to the initial submission, since a complete first submission is the fastest route to a decision.

- Clinical notes documenting the type 2 diabetes diagnosis.

- HbA1c lab result within the last 90 days.

- List of current and prior diabetes medications with dates.

- Prescriber name and NPI.

Incomplete requests are returned as "documentation incomplete". The return notice names each missing item so the prescriber can supply it without resubmitting the entire request. The review clock restarts when the missing items are received.

Clinical notes should be recent enough to reflect the member's present condition and should be signed or otherwise authenticated by the treating clinician. Scanned or faxed copies are acceptable when they are legible. Lab results should show the collection date, the reported value, and the name of the performing laboratory so that the reviewer can verify them.

Medication lists should be as specific as the prescriber's records allow. Each entry should show the drug name, the approximate start date, and the stop date if the drug is no longer in use. Where exact dates are not known, the month and year are sufficient. Gaps in the medication history should be explained in the notes whenever the prescriber is aware of the reason.

Prescriber identification is used to confirm that the request comes from a licensed clinician who participates with the plan. A request that lacks this identification cannot be matched to a prescriber record and will be handled as an incomplete submission.

Step Therapy

No step therapy requirement applies to Ozempic or Trulicity under this policy. The member is not required to try and fail another medication before a GLP-1 agent can be approved, and reviewers do not deny a request because a particular earlier drug was not used.

Metformin history is reviewed and noted when available. This review is informational. It helps the reviewer understand the member's treatment course, and the outcome of that review does not change the decision on the request. When the medication list does not mention metformin, reviewers record that no metformin history was found and continue with the remaining criteria.

Reviewers may also note other information relevant to the member's treatment course, such as the length of time on prior therapy, but these notes are for the record only and are not used as approval conditions.

Quantity Limits

The plan limits the amount of each medication that may be dispensed in a single fill period. The limits are set so that they match the usual labeled dosing schedules for a standard supply.

- Ozempic: 1 pen per 28 days.

- Trulicity: 4 pen doses (one carton) per 28 days.

Requests for higher quantities need a documented clinical reason. The prescriber should explain in the clinical notes why the standard limit does not meet the member's needs and should state the exact quantity requested. Reviewers evaluate such requests individually and may approve a higher limit for a defined period when the documentation supports it.

Early refill requests are subject to the plan's general refill timing rules, which are described in the member handbook rather than in this policy. Pharmacies that receive a rejection for quantity should contact the prescriber, who can submit the request for a higher limit through the same channel used for the original PA.

Lost, stolen, or damaged supplies are handled through a separate override process and are not governed by these limits. Members should speak with their pharmacy or call the plan's member services line for help with these situations.

Renewal

Approvals last 12 months from the approval date. The approval notice states the exact end date, and members are encouraged to ask their prescriber to begin the renewal process well before that date so that therapy is not interrupted.

Renewal needs clinical notes showing continued need and an HbA1c result from the last 90 days. A renewal request is reviewed against the criteria in this policy that are in effect on the date the renewal is received, which may differ from the criteria that applied to the original approval.

Renewal requests that arrive after an approval has ended are treated as new requests. In that situation the full documentation described earlier in this policy applies, and a gap in coverage may occur while the new request is under review.

This is a fictional policy created for software demonstration only.